



Vintage Optical

Number and methodology explainer

Morton, Illinois

July 29, 2026

Internal publish-candidate

Project Room review required

Public-only analysis

Confidence C

External actions: none

This document explains what every important number in the Vintage Optical analysis means, where it came from, how it was calculated, and what evidence could change it. It is an internal decision aid. Project Room status is inventory_review_required.

INTERNAL

How to read this analysis

The analysis uses four kinds of numbers:

- 1. **Raw public facts** are directly reported or observed values, such as the 2025 Morton village population estimate of **17,565**, the age shares of **25.9%** and **23.1%**, and the stored route duration of **244.3 seconds**.
- 2. **Directional bands** are evidence judgments on a five-value scale: **20, 35, 50, 65, 80**. A band translates incomplete public evidence into a consistent input. It is not a raw measurement.
- 3. **Derived scores** combine bands using fixed weights, preserve full precision through the calculation, and round once to the nearest whole point. A total ending in .50 rounds up.
- 4. **Structural numbers** organize the report or define a method. Examples include section 1/2/3, Tier 1/2, Fix Card IDs F-001/F-002/F-003, the 100-point display scale, a 30-day measurement window, the street number 605, and the 2026 copyright year. These are not performance measurements.

All scores displayed as report results are higher = better. The internal Competitive Pressure input is the one high = more pressure measure. It is not presented as a report score; it is inverted into the high-good Room to Win score.

What a neutral unknown 50 means

A **neutral unknown 50** is missing-evidence handling. It does not mean average performance was measured. It does not mean good, bad, or zero. It keeps an absent input at the midpoint while the rationale names the missing proof. Some 50 bands represent a neutral mixed public signal rather than a wholly absent fact, but they still make no favorable performance claim.

Number map

Number family	Value	Kind	Plain-language meaning
Client Opportunity	54 / 100	Derived score	How actionable the owner upside appears under current public evidence
Market Demand-Supply	57 / 100	Derived score	Whether the local market looks attractive enough to investigate
Competitive Pressure	57	Internal calculation input	Directional pressure, where higher means more pressure
Room to Win	43 / 100	Derived score	High-good inversion of pressure: 100 - 57 = 43
Practice Competitiveness	58 / 100	Derived score	Public-facing strength versus the bounded peer set
Digital Presence	57 / 100	Derived score	Findability, trust, booking, site, content, and local-proof read
Dry eye / ocular surface	52 / 100	Derived specialty score	RESEARCH NEXT
Myopia management	52 / 100	Derived specialty score	RESEARCH NEXT
Specialty contact lenses	51 / 100	Derived specialty score	RESEARCH NEXT
Current city context	17,565	Raw public fact	Official 2025 Morton village PEP estimate from S17
Historical city context	17,557	Frozen historical fact	Official 2024 estimate from the saved 2024-vintage PEP file
Under age 18	25.9%	Raw public fact	Morton village age share from S14
Age 65 or older	23.1%	Raw public fact	Morton village age share from S14
Focus On Eyes route	4.07 routed minutes	Derived route display	244.3 seconds / 60 = 4.0717 , rounded to 4.07, from R01
Catchment method	20-minute primary; 30-minute extended	Method parameter	Exurban comparison windows, not measured population polygons
Peer tiers	exactly 3 Tier 1; exactly 3 Tier 2	Structural count	A bounded comparison set, not a complete supply census
Action structure	exactly 3 Fix Cards; exactly 3 specialty lanes; one table	Plan count	The do-now measurement package
Measurement period	30 days	Plan parameter	Baseline window, not a forecast horizon

Headline scores

Band and calculation rules

The only allowed base bands are **20, 35, 50, 65, and 80**. Each weighted score uses:

`contribution = band or required computed input × weight`

The contributions are added at full precision, then rounded once. All weights within each score sum to 1.00. Client Opportunity correctly uses the already computed Market Demand-Supply result of 57 as one input. Room to Win correctly uses the already computed Competitive Pressure result of 57.

Market Demand-Supply: 57 / 100

headline-market-57

Question answered: Does the market look attractive enough to investigate?

Overall read: Mixed market. Higher = better.

Component	Band	Weight	Contribution	Why this band is logical	Sources	What remains unknown
Demand Strength	65	0.25	16.25	Morton village has directionally supportive under-18 and 65-plus shares. City facts support a demand hypothesis but are not catchment measurements.	S14 , S15 , S17	Catchment population, VDU, realized demand
Supply Balance	50	0.20	10.00	Neutral unknown because the named alternatives are not a complete geocoded and deduplicated supply set.	R01 , R02 , R03 , R04 , R05 , R06	Complete offices, providers, weighted supply, VDU per office
Payer / Income Fit	65	0.15	9.75	Morton village income is a favorable public proxy. A proxy supports a directional band, not an operating claim.	S14 , S15	Payer mix, commercial coverage, collections
Growth / Future Demand	50	0.15	7.50	Neutral unknown because no catchment growth series or forward growth evidence was collected.	S14 , S15	Permits, residential pipeline, school growth, mobility
Specialty Demand Load	65	0.15	9.75	City age shares directionally support pediatric, senior, and chronic eye-care need.	S14 , S15	Catchment clinical burden, search demand, contact-lens demand, referrals
Market Data Confidence	35	0.10	3.50	City facts and point routes are reproducible, but the fixed-window market evidence needed for a strong read is missing.	S14 , S15 , S17 , R01 , R02 , R03 , R04 , R05 , R06	All fixed-window demographics, VDU, supply, providers, patient choice

Full-precision calculation

$$\begin{aligned}
 &0.25 \times 65 + 0.20 \times 50 + 0.15 \times 65 + 0.15 \times 50 + 0.15 \times 65 + 0.10 \times 35 \\
 &= 16.25 + 10.00 + 9.75 + 7.50 + 9.75 + 3.50 \\
 &= 56.75
 \end{aligned}$$

Nearest whole point: **57 / 100**.

What could move it: Measured 20- and 30-minute isochrones joined to block-group demographics, a complete office and provider census, catchment VDU, growth evidence, and payer or collections summaries could move the result in either direction.

Competitive Pressure: internal input 57

headline-pressure-57

Question answered: How much directional pressure may realistic patient alternatives create?

Direction: Higher = more pressure. This internal input is inverted before report display.

Component	Band	Weight	Contribution	Why this band is logical	Sources	What remains unknown
Supply Saturation	50	0.40	20.00	Neutral unknown because the peer list is not a complete supply census and has no office or provider denominator.	R01 , R02 , R03 , R04 , R05 , R06	Complete supply, providers, population and VDU denominators
Patient Choice Pressure	65	0.25	16.25	Three named alternatives route within 20 minutes and five within 30 minutes, including a direct peer at 4.07 minutes and a retail substitute at 14.00 minutes.	R01 , R02 , R03 , R04 , R05	Patient origins, population weighting, full choice set, live traffic
Competitor Strength	65	0.20	13.00	Official pages show routine, medical, optical, dry-eye, and myopia overlap. This proves public offers, not patient draw.	S07 , S08 , S09 , S10 , S11 , S12	Peer utilization, reputation normalization, conversion, capacity
Access / Differentiation Pressure	50	0.15	7.50	Neutral mixed: Vintage publishes useful access and breadth, while peers publish overlapping access and services.	S02 , S06 , S07 , S08 , S11	Live slots, completion, call performance, comparable peer access

Full-precision calculation

$$\begin{aligned}
 &0.40 \times 50 + 0.25 \times 65 + 0.20 \times 65 + 0.15 \times 50 \\
 &= 20.00 + 16.25 + 13.00 + 7.50 \\
 &= 56.75
 \end{aligned}$$

Nearest whole point: **57 internal pressure**.

What could move it: A complete office/provider census, patient-origin choice analysis, independent peer-reputation data, and live access comparisons could raise or lower pressure.

Room to Win: 43 / 100

headline-room-43

Question answered: After translating pressure into the common higher-is-better direction, how much room appears available?

Room to Win = 100 - internal Competitive Pressure
100 - 57 = 43

Result: **43 / 100**, labeled tight. Higher = better.

This is an exact inversion, not a separately banded score. If better evidence moves Competitive Pressure up, Room to Win moves down by the same number of points, and vice versa.

Practice Competitiveness: 58 / 100

headline-practice-58

Question answered: How strong does Vintage Optical's public face look against the bounded peer set?

Overall read: Mixed. Higher = better.

Component	Band	Weight	Contribution	Why this band is logical	Sources	What remains unknown
Visibility vs Peers	50	0.20	10.00	Neutral unknown. A dated query shows Vintage and a booking action but is not a rank grid or complete result set.	S16	Peer-normalized rank and complete query coverage
Access / Booking vs Peers	65	0.20	13.00	The practice publishes Monday-Saturday hours, same-day emergency access, a prominent CTA, and a flow reaching service/provider selection.	S02 , S06 , S16	Live slots, completion, conversion, comparable peer friction
Review Trust vs Peers	50	0.20	10.00	Neutral unknown. The 4.9 sample lacks count, recency, responses, and equivalent peer fields.	S16	Peer-normalized reputation and review behavior
Service-Line Differentiation	65	0.15	9.75	Vintage publishes broad routine, medical, specialty, contact, and optical services, but peers overlap on key lanes.	S01 , S03 , S04 , S05 , S08 , S09 , S10	Lane conversion, outcomes, patient preference, ownership
Website / Conversion Clarity	65	0.15	9.75	Service pages, providers, location, hours, and appointment access are visible.	S01 , S02 , S03 , S04 , S05 , S06	Owned conversion, mobile performance, speed
Location Convenience	50	0.10	5.00	Neutral mixed. Parking and a current Main Street location are public, but a direct peer is 4.07 minutes away.	S02 , R01 , N00	Patient-origin convenience and trip tolerance

Full-precision calculation

$0.20 \times 50 + 0.20 \times 65 + 0.20 \times 50 + 0.15 \times 65 + 0.15 \times 65 + 0.10 \times 50$
 $= 10.00 + 13.00 + 10.00 + 9.75 + 9.75 + 5.00$
 $= 57.50$

Half-up rounding: **58 / 100**.

What could move it: A rank grid, independent peer review export, live appointment audit, booking funnel, mobile/site testing, and patient-origin convenience analysis.

Client Opportunity: 54 / 100

headline-opportunity-54

Question answered: How much actionable upside is visible to the practice owner?

Overall read: At risk under the canonical band label. This is not a grade. Higher = better.

Component	Value	Weight	Contribution	Why this value is logical	Sources	What remains unknown
Computed Market Demand-Supply	57	0.20	11.40	The canonical formula requires the computed market result, not a new manual band.	S14 , S15 , S17 , R01 , R02 , R03 , R04 , R05 , R06	See Market Demand-Supply unknowns
Competitive Pressure Opportunity	50	0.20	10.00	Neutral unknown. Pressure exists directionally, but no weakness cluster, leakage, or capacity-supported white space is proven.	S07 , S08 , S09 , S10 , S11 , S12 , R01 , R02 , R03 , R04 , R05	Competitor weakness, leakage, capacity-supported wedge
Practice Differentiation Upside	50	0.20	10.00	Neutral measure-first position. The offer is broad and key lanes overlap, so proof should precede expansion claims.	S01 , S03 , S04 , S05 , S08 , S09 , S10	Lane conversion, defensible ownership, unmet need
Access Fixability	50	0.15	7.50	Neutral unknown. Public access exists, but the analysis did not measure access failure.	S02 , S06	Calls, slots, abandonment, cancellations, no-shows
Digital Visibility Fixability	65	0.15	9.75	Existing public surfaces make baseline measurement and reconciliation a bounded, plausible action.	S01 , S02 , S03 , S04 , S05 , S06 , S16	Rank, review, directory, response, and channel baselines
Execution Simplicity	50	0.10	5.00	Neutral unknown. The first actions are bounded, but ownership and staff capacity are not evidenced.	S01 , S02 , S06	Owner, operating cadence, execution capacity

Full-precision calculation

$$0.20 \times 57 + 0.20 \times 50 + 0.20 \times 50 + 0.15 \times 50 + 0.15 \times 65 + 0.10 \times 50$$
$$= 11.40 + 10.00 + 10.00 + 7.50 + 9.75 + 5.00$$
$$= 53.65$$

Nearest whole point: **54 / 100**.

What could move it: Evidence of actual demand leakage, competitor weakness, specialty conversion, available capacity, access failure, clear ownership, and execution capacity.

Digital Presence: **57 / 100**

headline-digital-57

Question answered: Can a prospective patient find, trust, and book the practice from its public digital surface?

Overall read: Mixed. Higher = better.

Component	Band	Weight	Contribution	Why this band is logical	Sources	What remains unknown
Findability	50	0.25	12.50	Neutral unknown. The dated Maps sample shows Vintage but is not a rank grid or complete local-pack measure.	S16	Query coverage, rank distribution, directory consistency
Reputation	50	0.20	10.00	Neutral unknown. The 4.9 sample lacks count, velocity, recency, responses, and peer comparison.	S16	Peer-normalized reputation
Bookability	65	0.20	13.00	A public CTA reaches service and provider selection, and the Maps sample shows Book online.	S06 , S16	Slots, completion, abandonment, conversion
Site Quality	65	0.15	9.75	The site exposes location, hours, providers, and service-specific pages.	S01 , S02 , S03 , S04 , S05	Mobile rendering, Core Web Vitals, speed, conversion
Content & Specialty Signal	65	0.10	6.50	Dedicated dry-eye, myopia, and specialty-contact pages create a clear signal, though peers overlap.	S03 , S04 , S05 , S08 , S09 , S10	Search performance and unique lane ownership
Social / Local Proof	50	0.10	5.00	Neutral unknown. No verified social activity, local links, community mentions, or directory comparison were collected.	S01 , S16	Activity, local authority, community proof

Full-precision calculation

$$0.25 \times 50 + 0.20 \times 50 + 0.20 \times 65 + 0.15 \times 65 + 0.10 \times 65 + 0.10 \times 50$$
$$= 12.50 + 10.00 + 13.00 + 9.75 + 6.50 + 5.00$$
$$= 56.75$$

Nearest whole point: **57 / 100**.

What could move it: A repeatable rank grid, complete independent review data, tracked booking completion, mobile and performance tests, directory reconciliation, and verified social/local proof.

Specialty opportunities

All three specialty scores use the same canonical formula:

$$0.20 \times \text{Local Demand Fit} + 0.15 \times \text{Competitive Gap} + 0.15 \times \text{Current Capability} + 0.15 \times \text{Access/Capacity Fit} + 0.15 \times \text{Revenue/Reimbursement Potential} + 0.10 \times \text{Referral Ecosystem Fit} + 0.10 \times \text{Evidence Confidence}$$

A score from 50 to 64 means **RESEARCH NEXT** under the rubric. It does not forecast patients or revenue, declare white space, or recommend unmeasured expansion.

Dry eye / ocular surface: **52 / 100**

lane-dry-eye-52

Component	Band	Weight	Contribution	Why assigned	Sources	Unknown
Local Demand Fit	65	0.20	13.00	City age context supports a directional demand hypothesis.	S14 , S15	Catchment clinical and search demand
Competitive Gap	35	0.15	5.25	Tri-County and Vision Care Center publish direct dry-eye offers, so the lane is contested.	S08 , S09	Comparative depth, draw, outcomes
Current Capability	65	0.15	9.75	Vintage publishes a dedicated service and modalities.	S03	Volume, outcomes, staffing, throughput
Access / Capacity Fit	50	0.15	7.50	Neutral unknown.	S03 , S06	Slots, chair time, staff, rooms, follow-up
Revenue / Reimbursement Potential	50	0.15	7.50	Neutral unknown.	S03	Price, payer, collections, margin
Referral Ecosystem Fit	50	0.10	5.00	Neutral unknown.	S03	Relationships and referral density

Component	Band	Weight	Contribution	Why assigned	Sources	Unknown
Evidence Confidence	35	0.10	3.50	Offer and overlap are public; operating proof is limited.	S03 , S08 , S09 , S14 , S15	Demand, conversion, capacity, referrals, economics

$$13.00 + 5.25 + 9.75 + 7.50 + 7.50 + 5.00 + 3.50 = 51.50$$

Half-up rounding: **52 / 100 - RESEARCH NEXT.**

The lane is not a forecast or white-space claim because competitors publish overlapping offers and the analysis lacks catchment demand, conversion, capacity, referral, outcome, and economic evidence.

Myopia management: 52 / 100

lane-myopia-52

Component	Band	Weight	Contribution	Why assigned	Sources	Unknown
Local Demand Fit	65	0.20	13.00	The city under-18 share supports a directional family-demand hypothesis.	S14 , S15	Catchment, school, and search demand
Competitive Gap	35	0.15	5.25	Vision Care Center publishes several myopia-control approaches, so the lane is contested.	S09 , S10	Comparative depth, draw, outcomes
Current Capability	65	0.15	9.75	Vintage publishes a dedicated myopia-management service.	S04	Starts, retention, outcomes, capacity
Access / Capacity Fit	50	0.15	7.50	Neutral unknown.	S04 , S06	Follow-up capacity, chair time, staff skill, protocols
Revenue / Reimbursement Potential	50	0.15	7.50	Neutral unknown.	S04	Price, payment, retention, collections, margin
Referral Ecosystem Fit	50	0.10	5.00	Neutral unknown.	S04	School, pediatric, parent, provider referrals
Evidence Confidence	35	0.10	3.50	Offer, child-share proxy, and overlap are public; operating proof is limited.	S04 , S09 , S10 , S14 , S15	Conversion, capacity, referrals, economics

$$13.00 + 5.25 + 9.75 + 7.50 + 7.50 + 5.00 + 3.50 = 51.50$$

Half-up rounding: **52 / 100 - RESEARCH NEXT.**

It matches dry eye because both lanes currently have the same band pattern, not because the underlying services are identical.

Specialty contact lenses: 51 / 100

lane-specialty-contact-51

Component	Band	Weight	Contribution	Why assigned	Sources	Unknown
Local Demand Fit	50	0.20	10.00	Neutral unknown.	S05	Contact-lens population, keratoconus burden, search and referral demand
Competitive Gap	50	0.15	7.50	Neutral unknown. Peers publish contact-lens services, but equivalent specialty depth was not established.	S07 , S08 , S12	Complete specialty-depth comparison
Current Capability	65	0.15	9.75	Vintage publishes specialty lens types and an offer.	S05	Fit volume, success, chair time, capacity
Access / Capacity Fit	50	0.15	7.50	Neutral unknown.	S05 , S06	Fitting availability, doctor time, staff skill, rooms
Revenue / Reimbursement Potential	50	0.15	7.50	Neutral unknown.	S05	Pricing, payer, remakes, chair-time cost, margin
Referral Ecosystem Fit	50	0.10	5.00	Neutral unknown.	S05	Ophthalmology, optometry, keratoconus referrals
Evidence Confidence	35	0.10	3.50	The offer is public, but market and operating evidence is limited.	S05 , S07 , S08 , S12	Demand, gap, conversion, capacity, referrals, economics

$$10.00 + 7.50 + 9.75 + 7.50 + 7.50 + 5.00 + 3.50 = 50.75$$

Nearest whole point: **51 / 100 - RESEARCH NEXT.**

The result is not a forecast or white-space claim. A complete peer service audit and measured fitting demand, outcomes, capacity, referrals, and economics could move it.

Public facts and routes

Current and historical city population

population-vintages

- **17,565** is the official 2025 Population Estimates Program estimate for Morton village, Illinois, from **S17**. It is the current displayed **2025 city estimate**.
- **17,557** is the historical 2024 estimate from the frozen official 2024-vintage PEP file. The newer 2025-vintage file revises its own 2024 back-series value to 17,555, but that later revision does not erase the earlier file's historical lineage.
- Changing the current display from 17,557 to 17,565 changes no score or directional band. The scoring model did not substitute either city value into a drive-time catchment. The update adds current city context, but no new catchment population, VDU, supply, growth, or choice evidence.

Boundary: City context is not a drive-time catchment. Population, households, age bands, VDU, offices, providers, and ratios remain null for the 5-, 10-, 15-, 20-, and 30-minute windows.

Age shares

age-shares

- **25.9% under age 18** comes from Census QuickFacts **S14**. It is a city-level family-demand proxy.
- **23.1% age 65 or older** comes from **S14**. It is a city-level senior and medical-demand proxy.

The percentages are raw public facts. Their use in 65 bands is directional because the catchment distribution, realized care demand, payer mix, and patient behavior are unknown. The ages 18 and 65 define the Census categories; they are not separate performance measures.

Route and catchment method

route-and-catchment

The **R01** saved OSRM route from Vintage Optical to Focus On Eyes reports **244.3 seconds**:

244.3 seconds / 60 seconds per minute = 4.0717 minutes

Rounded to two decimals: **4.07 routed minutes**.

This is one practice-origin, point-to-point route. It has no live traffic or time-of-day model. It is not a polygon isochrone, patient-origin model, or complete supply census.

The market method uses a **20-minute primary** and **30-minute extended** window for the medium-confidence exurban classification. Exactly three named routed alternatives fall within the primary window and five within the extended window. These are bounded observations, not complete office counts. Illinois Eye Center has an official location source **S12** but no successful route **R06**.

The bounded peer set

Tier	Exactly three entries	Why the tier is structural, not a ranking
Tier 1 direct peers	Focus On Eyes; Tri-County Eye Center Washington; Vision Care Center Washington	Similar or overlapping optometry/service models judged relevant within nearby or extended context
Tier 2 substitutes	Bard Optical East Peoria; Illinois Eye Center Washington; Walmart Vision Center Morton	Different models that may substitute for part of demand

Tier 1 and Tier 2 are taxonomy labels. They do not mean first place or second place. The set was intentionally bounded for directional comparison and must not be read as the complete market.

Fix Card formulas

The report contains **exactly three GROW Fix Cards**, **exactly three existing specialty lanes**, and a **30-day** baseline window. F-001 calls for **one consolidated lane table**. These are plan counts, not measured results. Every formula is a plug-in measurement, not a forecast.

F-001 - Specialty conversion baseline

fix-f001

Applied separately to dry eye, myopia management, and specialty contact lenses:

inquiry-to-book rate = lane booked evaluations / lane inquiries

- Numerator: lane inquiries that produce a booked evaluation during the defined 30-day cohort.
- Denominator: all eligible, deduplicated inquiries assigned to that same lane and cohort.

book-to-start rate = lane starts / lane booked evaluations

- Numerator: patients who begin the lane's defined plan or service.
- Denominator: booked evaluations eligible to produce a start under the same cohort rule.

$$\text{completion rate} = \text{lane completed plans} / \text{lane starts}$$

- Numerator: starts meeting the practice's predeclared completion definition.
- Denominator: eligible lane starts with enough follow-up time under the stated cohort rule.

The one table should contain lane, inquiries, booked evaluations, starts, completed plans, available slots, and owner. Cohort timing, duplicates, cancellations, reschedules, and completion rules must be defined before calculation.

F-002 - Access diagnostic

fix-f002

$$\text{access gap count} = \text{unanswered or abandoned calls} + \text{booking attempts without completion} + \text{requests without an acceptable slot}$$

- Each term is an observed count under mutually exclusive disposition rules so one attempt is not counted twice.
- "Acceptable slot" needs a declared visit-type and time-window rule.

$$\text{realized gap rate} = \text{access gap count} / \text{total access attempts}$$

- Numerator: the deduplicated observed access gap count.
- Denominator: all eligible phone and digital access attempts in the same 30-day window.

$$\text{later sensitivity} = \text{measured access gap count} \times \text{tested recovery rate}$$

- First factor: a measured count, not an assumed leakage volume.
- Second factor: a recovery rate established by a controlled follow-up test, not a guessed conversion rate.

The output diagnoses whether a fixable gap exists. It does not assume leakage, patients, or revenue.

F-003 - Listing, review, referral, and recall baseline

fix-f003

$$\text{listing accuracy rate} = \text{correct tracked listings} / \text{tracked listings}$$

- Numerator: tracked public listings matching the declared name, address, phone, and other audited fields.
- Denominator: every surface included in the dated tracked-listing inventory.

$$\text{review velocity} = \text{new verified reviews} / 30 \text{ days}$$

- Numerator: new reviews meeting the predeclared verification and date rules.
- Denominator: the fixed 30-day observation period, not patient count.

$$\text{referral book rate} = \text{booked referred patients} / \text{referral inquiries}$$

- Numerator: eligible referred inquiries that produce a booking.
- Denominator: all eligible referral inquiries in the same cohort.

$$\text{recall book rate} = \text{booked recall patients} / \text{eligible recall contacts}$$

- Numerator: eligible recall contacts that produce a booking.
- Denominator: all contacts meeting the predeclared recall eligibility and outreach rules.

Zero denominator rule

For every rate, if the denominator is zero, missing, or not yet final, the result is **null / not calculable, never zero**. A zero rate would falsely claim that eligible opportunities existed and none converted. Counts missing from collection also remain null rather than being silently converted to zero.

Structural numbers and identifiers

Number or identifier	What it means	What it does not mean
Section 1, 2, 3	Navigation for market, competition, and opportunity	Score, rank, or priority magnitude
Tier 1 and Tier 2	Peer taxonomy classes	First place, second place, or measured quality
Exactly 3 Tier 1 and 3 Tier 2	Bounded peer-set design	Complete office or provider census
F-001, F-002, F-003	Stable Fix Card identifiers and order	Measured impact or importance

Number or identifier	What it means	What it does not mean
S01-S17, N00, R01-R06	Source registry identifiers	Values, scores, or evidence strength by their digits
100	Common score denominator and inversion base	Raw fact or percentage confidence
20 and 30 minutes	Selected primary and extended method windows	Measured isochrone populations
30 days	Baseline collection window	Forecast horizon or promised result
605	Street number in 605 S Main St	Count or score
2024 and 2025	Source-vintage and estimate years	Score values
2026	Copyright/report year	Evidence measurement
18 and 65	Age-category thresholds	Standalone market results
60	Seconds-per-minute conversion factor	Observed route value

Logical consistency checks

Why the scores can coexist

- **Market 57** says public age and income context is supportive, while supply, growth, VDU, and fixed-window market proof are incomplete.
- **Room to Win 43** says directional competitive pressure is tighter. It is exactly the inverse of internal pressure 57 and does not contradict Market 57. A market can have demand support and still contain meaningful alternatives.
- **Practice Competitiveness 58** says Vintage's public service breadth and access path are useful, while visibility, reputation, live access, conversion, and convenience are not peer-normalized.
- **Client Opportunity 54** combines the market result of 57 with four neutral 50 inputs and one 65 for bounded digital measurement fixability. It asks about actionable upside, not market attractiveness alone.
- **Digital Presence 57** reflects usable booking, site structure, and specialty content, offset by unmeasured rank, reputation, and social/local proof.

Why the population refresh changes no score

The historical display of 17,557 was updated to the current 2025 estimate of 17,565. Neither value is a measured 20- or 30-minute catchment population. The update therefore does not alter Demand Strength, Specialty Demand Load, Supply Balance, Market Data Confidence, or another base band. No full-precision total or rounded score changes.

Why values repeat

- Market Demand-Supply, internal Competitive Pressure, and Digital Presence each equal **57**, but they answer different questions and use different component sets. Each happens to total **56.75** before rounding.
- Dry eye and myopia both equal **52** because their current band patterns are identical: 65, 35, 65, 50, 50, 50, 35 under the same weights. The evidence behind those bands is lane-specific.
- **50** recurs because it is the neutral midpoint used for missing or mixed evidence. Repetition is not proof that measured performance is average.
- **65** recurs as a directionally supportive band where a public signal exists but stronger operating proof does not.
- **35** recurs where evidence confidence is limited or a specialty gap is publicly contested.
- **100** recurs only as the common score scale and Room to Win inversion base.

Disconfirmers

1. A complete geocoded office census and provider join could move pressure, supply balance, and Room to Win materially in either direction.
2. Patient-origin analysis, live traffic, and trip-tolerance evidence could change which peers are practically relevant.
3. A rank grid and equivalent peer review fields could change visibility, reputation, Practice Competitiveness, and Digital Presence.
4. Public pages may overstate or understate actual booking completion, conversion, capacity, outcomes, and economics.
5. The public specialty breadth already visible at Vintage disproves a simple "no offer" claim, while competitor overlap disproves an uncontested white-space claim.
6. Catchment demographics may differ from Morton village context.
7. Measured operating ownership and staff capacity could make the Fix Cards easier or harder to execute than the neutral 50 implies.

What we do not know

The public-only evidence does not establish:

- catchment population or households for any fixed drive-time window;
- complete eye-care supply or a deduplicated office census;
- cleaned OD/provider counts;
- Vision Demand Units, or VDU, and VDU per office;
- population-weighted patient choice;
- peer-normalized visibility or reputation;
- independent review count, recency, response behavior, and comparable peer fields;
- live appointment access, phone handling, slot depth, completion, cancellations, no-shows, or unmet requests;
- booking, inquiry, start, completion, recall, referral, or optical conversion;
- specialty capacity, staffing, chair time, rooms, protocols, and execution capacity;
- clinical outcomes or patient draw;
- referral relationships or referral density;
- payer mix, pricing, collections, margin, contribution, or other economics.

Confidence remains **C** because the analysis is public data plus a directional local and competitor scan. The missing evidence is why the specialty lanes remain RESEARCH NEXT and the Fix Cards begin with measurement.

Source dictionary: public sources

The registry contains **24 unique source IDs**: S01-S17, N00, and R01-R06. This first table contains the 17 public-source records, S01-S17. Each ID links to its registered public URL.

ID	Claim use	Confidence / status	Limitation
S01	Vintage identity, service breadth, on-site lab, three named optometrists, appointment CTA	High / fetched	Practice-controlled claims do not prove utilization, outcomes, capacity, conversion, or economics.
S02	Current address, hours, parking, same-day emergency statement, appointment link	High / fetched	Posted access does not prove live availability, phone performance, or realized demand.
S03	Public dry-eye service and modalities	High / fetched	Does not prove volume, outcomes, capacity, or contribution margin.
S04	Public myopia-management service and options	High / fetched	Does not prove starts, retention, outcomes, or economics.
S05	Public specialty-contact service and lens types	High / fetched	Does not prove referrals, chair time, fit success, capacity, or economics.
S06	Public booking path reached service and provider selection	High / fetched, no submission	No selection or submission occurred; slots and completion remain unknown.
S07	Focus On Eyes identity, Morton location, services, hours, appointment request	High / fetched	Peer-controlled content; no independent reviews, live slots, utilization, or capacity.
S08	Tri-County identity, location, broad services, dry-eye overlap	High / fetched	Public claims do not prove capacity, volume, outcomes, or Morton patient draw.
S09	Vision Care Center identity and dry-eye overlap	High / fetched	No independent utilization, outcomes, capacity, or patient-origin evidence.
S10	Vision Care Center myopia-control approaches	High / fetched	Public positioning does not prove starts, outcomes, capacity, or economics.
S11	Bard East Peoria identity, address, model, appointment access	High / fetched	Does not prove demand from Morton or peer-normalized reputation.
S12	Illinois Eye Center Washington identity, address, service breadth	High / fetched	Entity is current, but the supplied geocode produced no match or route.
S13	Attempted first-party Walmart Vision Center verification	Unknown / blocked	Bot-blocked; no official service, hours, booking, or review claim is treated as fetched.
S14	Morton QuickFacts age shares, income, density, and commute	High / fetched	Mutable city-boundary context, not a drive-time catchment.
S15	2024 ACS 5-year dataset authority and vintage	High / fetched	Documentation URL; no block-group-to-isochrone join was completed.
S16	Dated limited Maps sample showing 4.9 and Book online	Medium / partial sample	No reliable count, recency, responses, peer normalization, or complete result set; rating is not clinical proof.
S17	Official Morton village 2025 PEP estimate of 17,565	High / fetched and frozen	Village boundary only, not a 5-, 10-, 15-, 20-, or 30-minute catchment population.

Source dictionary: receipts and routes

The second table contains the seven geocode and route records, N00 and R01-R06. These IDs also link to their registered public URLs. The receipt manifest that follows lists package-relative audit files shipped in the report packet. Those files preserve the exact values used for refresh and audit; they are not external links.

ID	Claim use	Confidence / status	Limitation
N00	Exact-address subject geocode used as route origin	High / matched	OpenStreetMap geocoding is not an official practice listing and needs entity review.
R01	Vintage to Focus On Eyes: 4.07 minutes, 1.56 route miles	High / routed	One origin route, no traffic or time-of-day model, not an isochrone or choice set.
R02	Vintage to matched Walmart Supercenter: 5.71 minutes, 2.35 miles	Medium / routed	Match is the store entity, not a separately matched Vision Center; location context only.
R03	Vintage to Tri-County Washington: 20.94 minutes, 10.85 miles	High / routed	One origin route, no traffic, not an isochrone or patient-choice set.
R04	Vintage to Vision Care Center Washington: 22.41 minutes, 16.33 miles	High / routed	One origin route, no traffic, not an isochrone or patient-choice set.
R05	Vintage to Bard East Peoria: 14.00 minutes, 9.25 miles	High / routed	One origin route, no traffic, not an isochrone or patient-choice set.
R06	Attempted Illinois Eye Center Washington geocode	Unknown / geocode missing	No coordinate match or OSRM route; identity and address rely on S12.

Receipt manifest

These package-relative paths identify files shipped in the report packet:

- Current receipt summary: `data/receipt_summary.json`
- Frozen official 2025 PEP file: `data/source_receipts/census-pep-sub-est2025.csv`
- Frozen official 2024-vintage PEP file: `data/source_receipts/census-pep-sub-est2024.csv`
- Saved QuickFacts audit extract: `data/source_receipts/quickfacts-live-audit-extract.json`
- Saved current R01 response: `data/source_receipts/osrm-r01-current.json`

The registered source URLs remain the clickable public references. The shipped receipt files are package-local audit artifacts.

Final interpretation

Market Demand-Supply at **57 / 100** shows directional demand support, while Room to Win at **43 / 100** is the exact high-good inversion of internal Competitive Pressure 57 and therefore shows a tighter competitive opening. Practice Competitiveness at **58 / 100**, Client Opportunity at **54 / 100**, and Digital Presence at **57 / 100** describe different questions, so their nearby values are not duplicate findings. The unchanged specialty scores, dry eye **52 / 100**, myopia management **52 / 100**, and specialty contact lenses **51 / 100**, remain RESEARCH NEXT because the population refresh did not add catchment, conversion, capacity, outcome, referral, or economic proof. This is still an internal publish-candidate under the public-only evidence boundary. Project Room review remains required before any outward use.

Vintage Optical, Morton
Internal publish-candidate - Project Room review required - Public-only - Confidence C - External actions: none
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